Eero Care Platform

Paperless intake. Coordinated care.
A clearer workspace for clients and therapists.

FUNCTIONS, ARCHITECTURE & IMPLEMENTATION STATUS

Version 1.1 · October 10, 2026

Prepared for Eero Therapeutic Services

A modern care-management concept built around Eero's youth, family, and caregiver services. This document describes the client portal, administrator workspace, therapist workspace, document capture, AI review workflow, and development backend.

Current release

Interactive browser-local prototype + development backend + tested sample-data AI adapter. The hosted preview has no live clinical database or AI connection.

Clinical and compliance boundary

This release is for fictional demonstration data. It is not certified as HIPAA compliant, medically approved, or suitable for real patient records. AI produces reviewable drafts and extraction candidates; a licensed clinician remains responsible for assessment, diagnosis, and care decisions.

Original Eero identity and interface. No affiliation with Epic or other vendors.

Purpose, brand, and focused workspaces

Eero Therapeutic Services supports youth and families through Intensive In-Community services, Behavioral Assistance, Parent Coaching, assessments, and group programs. The supplied company document identifies Eero as a Yoruba word associated with calm. The product carries that idea into a simple intake experience and a focused care workspace.

Brand system

An interlocking “e” symbol pairs plum and sage with a clear wordmark. Plum anchors navigation and actions; sage marks supportive accents; warm pale surfaces create separation. Neutral colors avoid a gender-coded overall app identity. The interface includes a dark theme and responsive layout.

Audience	Current experience	Production responsibility
Client / caregiver	Simple home; intake; document uploads; request status	Verified identity, guardian authority, clinical consent and accessible self-service
Administrator	Intake inbox; source review; service routing; staff assignment; tasks	Organization-scoped access, policy-driven assignment, operational oversight
Therapist	Briefings; cases; chart navigator; notes; encounters; goals	Assigned-case access, clinical review, approved documentation and follow-up

Company program catalog

Connection Crew (6–8), Uplift Squad (9–11), EmpowerHer and EmpowerHim (12–14 and 15–17), From Grief to Grace, and Bond Builders appear as program references and enrollment options. These descriptions inform navigation; enrollment does not establish eligibility or clinical appropriateness.

Design distinction

The app uses familiar clinical concepts such as a longitudinal chart, reported medications and allergies, encounters, and structured notes. These functions are implemented with original Eero presentation. Vendor software, private designs, proprietary content, and branding are not reproduced.

The entry screen previews the client experience after sign-in, with a separate staff link. It does not authenticate accounts. Hiding interface elements does not enforce access control.

Paperless intake and the handoff to care

1 / Introduce the client

The client or caregiver enters a fictional client name, age group, relationship, contact name, email, and optional phone. The form collects a preferred contact method and validates required fields.

2 / Describe the need

The client reports concerns, duration, goals, and a service preference. “Help me choose” is available. The system preserves these as reported statements rather than interpreting them as a diagnosis.

3 / Capture or attach

The client can select a photo or PDF, and compatible mobile devices can offer camera capture. Documents can also be added from the client home after an intake is submitted. The preview limits attachments to 15 MB each. The general document workspace supports capture, drag-and-drop, thumbnails, classification, and review. Actual capture behavior depends on the device and browser.

4 / Review and submit

A review step and fictional-data acknowledgment precede submission. The receipt supplies a reference number. The My request section shows Submitted, In review, or Assigned status for submissions created through this browser’s client preview. The intake enters a browser-local inbox with New, In review, and Assigned stages. This acknowledgment is not clinical consent or a legally validated electronic signature.

5 / Administrator review and placement

An administrator reviews contact details, concerns, goals, and source files; chooses a provisional service and a demonstration therapist; and confirms source review. Assignment creates a case, an intake document, linked attachment records, and a review task. Reassignment updates the existing handoff instead of creating duplicate cases.

6 / Therapist preparation

The assigned intake supplies reported concerns and goals, neutral questions, missing-context prompts, and a preparation checklist. The hosted briefing is assembled from intake fields and explicitly labeled as not AI-generated. It is not a treatment plan.

Where the records currently live

Intakes, cases, documents, and assignments persist in IndexedDB in the current browser. They do not synchronize with another browser or the Python database. A production ingestion API, authenticated submissions, encrypted object storage, and durable server-side routing remain required.

The demonstration is not monitored for emergencies. The intake shows U.S. 911 and 988 guidance. Automated urgency assessment and crisis response are not implemented.

Function inventory and release status

Function	Available now	Boundary / next step
Client intake and receipts	Interactive prototype	Fictional data; no server synchronization
Intake inbox and routing	Interactive prototype	Manual routing and demonstration staff
Cases and profiles	Interactive prototype	Browser-local records
Capture, attachments, document review	Interactive prototype	No hosted OCR or malware scanner
AI field extraction	Local API adapter	PNG/JPEG only; all fields require review
Therapist preparation brief	Template in preview; local AI adapter	No diagnosis or condition-specific treatment plan
Chart navigator and history	Interactive prototype	Manual entries; no external record feeds
Reported medications and allergies	Interactive prototype	Unverified; no prescribing or reconciliation
Dated observation flowsheet	Interactive prototype	Manual entries; no diagnostic scoring
Encounter records	Interactive prototype	Links to draft notes
SOAP / DAP / general notes	Interactive prototype	Drafts; no verified e-signatures
Care goals and progress	Interactive prototype	Manual objectives and progress
Tasks, calendar, program enrollment	Interactive prototype	No external calendar or payer integration
Insights, search, dark theme, exports	Interactive prototype	Demonstration metrics; no population claims
Authorization, encrypted fields, audit	Development service	Local tests; not hosted authorization
Large clinical database structure	PostgreSQL blueprint	Not deployed or performance-tested
Billing, claims, interoperability	Roadmap	No live claims, e-prescribing, FHIR or HL7

“Interactive prototype” means the controls change browser-local sample state. “Local API adapter” means a real provider call can be run from the development backend; it does not imply that the hosted preview calls that provider.

Clinical workspace and AI assistance

An organized clinical chart

Each case has overview, dated observations, medications, allergies, encounters, and history tabs. Care goals capture objectives, measures, target dates, and manually selected progress. Encounter entries record service, date, and location and can link to draft documentation.

Notes and clinical accountability

General service notes, SOAP, and DAP templates support structured documentation. Notes remain drafts pending review. An empty allergy list is described as unknown or not reviewed, rather than “no known allergies.” Reported medication and allergy entries retain a source and remain unverified. No medication ordering, drug interaction screening, or treatment recommendation engine is implemented.

Image-to-data assistance

The local AI adapter sends an explicitly confirmed fictional PNG/JPEG to the OpenAI Responses API. It returns candidate field names, visible values or null, source excerpts, unreadable sections, and limitations using a structured schema. It records a source hash, forces review for every field, and prevents automatic chart application. PDFs are accepted in the preview as attachments but are not supported by this extraction adapter.

Therapist briefing assistance

The local adapter accepts selected verified intake fields, excluding contact fields, and drafts a reported summary, missing information, and neutral assessment questions. It marks the result for clinician review and states that no diagnosis has been established. A separate general NIMH psychotherapy resource supplies orientation; it is not individualized guidance.

Clinical boundaries and review design

The AI prompt directs the model to treat document text as data, avoid invented facts, and avoid diagnosing, prescribing, assigning severity, or determining emergency safety. These prompt constraints cannot guarantee model behavior. Production requires clinical evaluations, adversarial testing, source verification, escalation procedures, and enforcement beyond the prompt.

Verified integration status

A live fictional intake briefing and a synthetic fictional intake-image extraction passed. Unit tests cover sample-data enforcement, request configuration, structured results, and review flags. The hosted preview clearly labels its extraction example as a sample and its intake briefing as template-based.

A provider key exists only in an ignored local environment file. It is excluded from source, downloadable packages, and client code. store:false is set for the request; this setting alone does not establish HIPAA suitability or provider data-retention terms.

The backend “brain” and database foundation

The backend brain is a combination of structured records, access decisions, workflow rules, and reviewable AI adapters. It is not an autonomous clinical decision-maker. The development service uses SQLite to demonstrate rules; a separate PostgreSQL blueprint proposes the larger production data model.

Layer	Development implementation	Production requirement
Identity and authorization	Organization, role and case-assignment service checks	Trusted identity provider, MFA, sessions, server-derived actor context
Clinical records	Cases, notes, documents, tasks and authorization metadata	Managed encrypted database, migrations and record lifecycle
Document storage	Validated types; quarantine/scan states	Private encrypted object store; trusted scanning; signed access
Audit	Append-only triggers and hash chain	Independent tamper-resistant retention and monitoring
Workflow rules	Pending review, overdue tasks, expired authorizations	Durable jobs, retries, idempotency and observability
AI assistance	Fictional-data extraction and brief adapters	Approved provider terms, evaluation and clinical governance

Data domains

The PostgreSQL blueprint covers organizations, staff, clients and guardians, cases and assignments, encounters, versioned documents, progress notes, separately protected psychotherapy notes, tasks, authorizations, programs, and enrollments. Row-level security is forced with a fail-closed starting point; deployment policies and trusted session context still need implementation and testing.

Sensitive records and files

The development service encrypts selected fields with AES-GCM and separates psychotherapy-note access by author. It checks organization and assignment boundaries. Validated PDF/JPEG/PNG uploads enter quarantine and remain unreadable until a trusted scanner marks them clean. These controls are service examples, not a production identity or storage deployment.

Capacity and recoverability

No patient-count, concurrency, uptime, or storage-capacity guarantee is established. Production sizing needs workload measurements, indexes, migration tests, backup and restore exercises, recovery objectives, and database isolation checks. Some development metadata is unencrypted; full database and backup encryption remain necessary.

The PostgreSQL blueprint has not been applied to a hosted database. The frontend is not connected to this service.

Privacy, security, and production readiness

HIPAA compliance requires appropriate organizational practices, contracts, and operation of the complete system. A large database or encryption feature alone is insufficient. This document describes a design direction and does not claim regulatory approval.

Required before real-patient ingestion

• Complete a documented security risk analysis, remediation, organizational policies, staff training, and privacy review.

• Establish appropriate business associate agreements for hosting, storage, backups, logging, communications, and any AI provider handling protected health information.

• Connect trusted authentication, MFA where appropriate, server-side role and case authorization, session expiry, tenant isolation, and access revocation.

• Deploy private encrypted document storage, trusted malware scanning, key management, access logs, retention policies, and verified restoration.

• Implement clinically approved intake/consent forms, guardian authorization, signature requirements, record access and amendment workflows, and disclosure controls.

• Review psychotherapy-note protections and other applicable state, federal, contractual, and program requirements for the actual service population.

• Complete clinical AI evaluation, review and correction flows, source traceability, failure handling, and approved escalation procedures.

Operational controls to implement

Production needs rate limits, file and content limits, dependency maintenance, secret rotation, incident response, monitoring, audit retention, and backup verification. Patient information should not enter ordinary telemetry or application error logs. Data sent to third-party providers must follow approved privacy and contractual arrangements.

Clinical decision-support review

FDA guidance distinguishes software functions based on their intended use and whether professionals can independently review the basis for recommendations. Any future diagnostic or treatment recommendation feature requires a specific clinical and regulatory assessment. This product has not been determined to qualify for an exemption or received FDA clearance.

Demonstration safeguards

The preview repeatedly requests fictional data. Hosted AI is not connected, role buttons are labeled as previews, and local AI calls require a sample confirmation flag. These safeguards do not make browser-local storage suitable for clinical records.

Security, clinical, accessibility, and regulatory reviews must assess the entire deployed workflow rather than relying on feature labels.

Validation, roadmap, and primary references

Validation performed

24 backend tests passed: 18 core service checks and 6 AI adapter checks. Four frontend flow tests also passed for client request filtering, invalid uploads, assigned-case document routing, and storage-failure rollback. Live fictional briefing and image-extraction smoke checks passed. JavaScript syntax and source consistency checks were performed. No browser usability test, mobile-camera validation, penetration test, independent compliance assessment, hosted PostgreSQL test, or backup restoration exercise was completed.

Delivery roadmap

Phase	Work	Exit evidence
1 · Experience	Client intake, assignment, charts, documents, goals, accessible layout	Observed browser and phone workflows; stakeholder review
2 · Secure records	Identity, production database, storage, scans, consent and audit	Boundary tests, risk review, contracts and restore evidence
3 · Reviewed AI	Extraction review, factual briefings, provenance, evaluations	Clinical acceptance criteria, failure controls and measured quality
4 · Interoperability	Approved record exchange, scheduling and billing integrations	Vendor agreements and end-to-end integration verification

Source register

Company context: user-supplied “Eero Templets.docx”; founder and program descriptions. User-supplied Epic Study Notebook saved source: flowsheet, checklist, and role-access learning examples; used as conceptual inspiration, not clinical policy.

1. HHS · HIPAA and cloud computing — primary source

2. HHS · Security Rule summary — primary source

3. HHS · Mental health and psychotherapy-note protections — primary source

4. FDA · Clinical decision-support assessment — primary source

5. NIMH · Psychotherapies — primary source

6. OpenAI · Images and vision — primary source

7. OpenAI · Structured outputs — primary source

References inform the design; they do not verify this implementation. Status statements describe the October 10, 2026 development release.